Community Supports (CS)
Provider Application

Credentialing Application

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APPLICATION CHECKLIST

All information must be completed in full, signed and dated by Applicant.
Indicate any areas that Do Not Apply with N/A.

All items below MUST be checked accordingly, in order for the Credentialing package to be accepted.

- ☐ Credentialing Application ALL sections: Complete and Legible.
- ☐ Explanation(s) for any "Yes" answer of questionnaire.
- ☐ General Provisions of application: Signed and Dated.
- ☐ A copy of applicable state licenses and/or a copy of current business license: Current Copy.
- ☐ A copy of general/professional liability coverage
- ☐ Disclosure of any history of liability claims in the past 7 years (if applicable)
- ☐ A Copy of accreditation/certification (if applicable, not required)
- ☐ IRS Form 990 or other documentation showing nonprofit status (if applicable, not required)

CREDENTIALING APPLICATION

SECTION I: FACILITY DEMOGRAPHIC INFORMATION

☐ Initial Credentialing ☒Re-Credentialing

*Please complete the below section *(Please Print in Black Ink)*

Legal Business Name (as reported to the IRS):	Doing Business As (DBA) Name (if applicable):
Blue Horizon Community Health Services, Inc.	Blue Horizon Care Network
Facility Type (Please check all that apply):	**Community Supports Provided (Please check all that apply):**
Community Health Center Organization Serving the Unhoused Other: Community support services provider	Housing Transition Navigation Services Housing Tenancy and Sustaining Services Housing Deposits Recuperative Care (Medical Respite) Personal Care and Homemaker Services

Organizational Information:

Contact Person:	Contact Job Title:	Contact Phone #:	Contact Email:
Aisha Patel	Program Director	(555) 310-2400	aisha.patel@bluehorizon.example.org
Credentialing Contact:	Job Title:	Credentialing Phone#:	Credentialing Contact Email:
Michael Torres	Credentialing Manager	(555) 310-2415	credentialing@bluehorizon.example.org

Are you enrolled in Medi-Cal? ☒Yes ☐No	Are you enrolled in Medicare? ☒Yes ☐No

NPI #:	Medi-Cal #:	Medicare #:	Federal Tax ID:
1987654321	MC-472981	PTN-884211	84-3179265

Race/Ethnic Group: (Optional):	Electronic Medical Records
Asian ☒White ☒Hispanic ☒Other: Multiracial leadership team	☐ Yes ☐ No

Languages: (Identify any foreign language(s) or sign language spoken fluently when working with patients)

☐ Arabic (AR)	Spanish (SP) ☒	☐ Japanese (JA)	Hindi (HI) ☒	Other: Punjabi
☐ German (GE)	☐ Vietnamese (VI)	☐ Portuguese (PO)	Korean (KO) ☒	
☐ Italian (IT)	☐ Chinese (CH)	☐ Farsi (FA)	☐ Russian (RU)	
☐ Laotian (LA)	☐ Hebrew (HE)	☐ Sign Language (SL)	☐ Tagalog (TA)	

SECTION I: FACILITY DEMOGRAPHIC INFORMATION (continued)

Parent / Main Organization Information:					
Legal Business Name Blue Horizon Community Health Services, Inc.			Legal Business Name Blue Horizon Care Network		
Street Address 2450 Westlake Avenue	Suite # Suite 410		Street Address 2450 Westlake Avenue	Suite # Suite 410	
City Sacramento	State CA	Zip 95815	City Sacramento	State CA	Zip 95815
Phone (555) 310-2400	Fax (555) 310-2401		Phone (555) 310-2400	Fax (555) 310-2401	
Email: info@bluehorizon.example.org			Email: operations@bluehorizon.example.org		

Practice Locations		
Address Information (*Please list all locations and group affiliations. If needed, use 'Attachment A' enclosed.)*	2	_____

Primary Facility Location				Second Facility Location *(if applicable)*			
Legal Business Name Blue Horizon Community Health Services, Inc.				Legal Business Name Blue Horizon Housing Services			
Street Address 2450 Westlake Avenue			Suite# Suite 410	Street Address 880 Harbor Boulevard			Suite# 120
City Sacramento	State CA	Zip Code 95815	County Sacramento	City West Sacramento	State CA	Zip Code 95691	County Yolo
Phone (555) 310-2400	Fax (555) 310-2401	TTY Number 711		Phone (555) 310-2420	Fax (555) 310-2421	TTY Number 711	
NPI # (If different than above): 1987654321				NPI # (if different from above) 1987654322			
Federal Tax ID (if different from above): 84-3179265				Federal Tax ID (if different from above): 84-3179265			
Email: cs@bluehorizon.example.org				Email: housing@bluehorizon.example.org			
Translation Services Available ☒Yes ☐No				Translation Services Available ☒Yes ☐No			
Normal Business Hours 8:00 AM - 5:00 PM	Schedule: M ☒T ☒W ☒TH ☒F ☒			Normal Business Hours 8:30 AM - 5:30 PM	Schedule: M ☒T ☒W ☒TH ☒F ☒		

SECTION II: LICENSING/CERTIFICATION/ACCREDITATION

Facility License Information: (identify all licenses used to operate your office(s). Please return copies of each)			
Provider License Type *If applicable*	State of License	License No. *(Attach copy of license)*	Expiration Date
Community Clinic License	CA	CCL-204588	12/31/2026
Business License	CA	BL-775190	06/30/2026
Housing Support Program	CA	HSP-331204	09/30/2026

Facility Accreditation: (Please indicate if this location has been reviewed by any of the Accrediting authorities listed below and provide a copy of the most recent accreditation report.)	
☐ American Association for Accreditation of Ambulatory Surgical Facilities (AAAASF)	☐ Accreditation Association for Ambulatory Health Care (AAAHC)
☐ Accreditation Commission for HealthCare (ACHC)	☐ American Osteopathic Association (AOA)
☐ American Association of Diabetes Educators (AADE)	Not Applicable ☒
☐ Continuing Care Accreditation Commission (CCAC)	☐ Community Health Accreditation Program (CHAP)
☐ Clinical Laboratory Association Improvement Amendments (CLIA)	☐ Council on Accreditation for Children and Family Services (COA)
☐ Commission on Office Laboratory Accreditation (COLA)	☐ Det Norske Veritas National Integrated Accreditation for Healthcare Organization (DNV)
☐ Healthcare Facilities Accreditation Program (HFAP)	☐ Indian Health Service (HIS)
☐ The Joint Commission (TJC)	☐ Not Applicable

SECTION III: OWNERSHIP DISCLOSURE FORM

PROVIDER ENTITY STATUS (Check if Applicable)	MINORITY & OWNERSHIP CHARACTERISTICS (Check if Applicable)
Corporation-Not for Profit ☒	Asian American ☒ American Women ☒

List names and addresses of all principals and indicate percent ownership, if applicable ("Principal" means any shareholder, officer, partner, joint venture or anyone else having an ownership in or managerial control over the PROVIDER. Attach additional sheets if necessary.)

Sophia Reynolds	Chief Executive Officer	2450 Westlake Avenue, Suite 410, Sacramento, CA 95815	45%
Daniel Brooks	Chief Financial Officer	2450 Westlake Avenue, Suite 410, Sacramento, CA 95815	30%
Maya Chen	Chief Operations Officer	2450 Westlake Avenue, Suite 410, Sacramento, CA 95815	25%

SECTION IV – ATTESTATIONS

ATTESTATION QUESTIONS
Please answer the following questions "yes" or "no." If any answer to a question is "yes," please provide full details on separate sheet.
A. Has this organization, under any current or former name or business identity, ever had any felony or misdemeanor convictions, under Federal or State law, related to: (a) the delivery of an item or service under Medicare or State health care program, or (b) the abuse or neglect of a patient in connection with the delivery of a health care item or service? ☐ **Yes** ☐ **No**
B. Has this organization, under any current or former name or business identity, ever had any felony or misdemeanor convictions, under Federal or State law, related to theft, fraud, embezzlement, breach of fiduciary duty, or other financial misconduct in connection with the delivery of a health care item or service? ☐ **Yes** ☐ **No**
C. Has this organization, under any current or former name or business identity, ever had any felony or misdemeanor convictions, under Federal or State law, related to the interference with or obstruction of any investigation into any criminal offense described in Title 42 - Code of Federal Regulations Section 1001.1001 or 1001.201? ☐ **Yes** ☐ **No**
D. Has this organization, under any current or former name or business identity, ever had any felony or misdemeanor convictions, under Federal or State law, relating to the unlawful manufacture, distribution, prescription, or dispensing of a controlled substance? ☐ **Yes** ☐ **No**
E. Has the organization ever had the State license involuntarily denied, revoked, suspended, not renewed, placed under probation, subject to disciplinary action or otherwise limited or curtailed; or has the organization voluntarily relinquished the State license in anticipation of any of these actions; or are any of these actions pending with respect to the State license? ☐ **Yes** ☐ **No**
F. Has the organization ever been charged, suspended, fined, disciplined, or otherwise sanctioned, submitted to probationary conditions, restricted or excluded, or has the organization voluntarily relinquished eligibility to provide services or accepted conditions on its eligibility to provide services, for reasons relating to possible incompetence or improper professional conduct, or breach of contract or program conditions, by Medicare, Medicaid, or any public program, or is any such action pending? ☐ **Yes** ☐ **No**
G. Has the organization had its membership, contractual participation or employment by any medical organization (e.g. hospital medical staff, medical group, independent practice association (IPA), health plan, health maintenance organization (HMO), preferred provider organization (PPO), private payer (including those that contract with public programs), medical society, professional association or other health delivery entity or system), ever been denied, suspended, restricted, reduced, subject to probationary conditions, revoked or not renewed for possible incompetence, improper professional conduct or breach of contract, or is any such action pending? ☐ **Yes** ☐ **No**
H. Has the organization ever had any other regulatory agency (OSHA, etc.) deny, revoke, suspend, not renew, place under probation, subject to disciplinary action or otherwise limited or curtail operations; or are any actions pending from any other regulatory agency? ☐ **Yes** ☐ **No**
I. Has the organization ever had accreditation by an organization (CLIA, JCAHO, etc.) involuntarily denied, revoked, suspended, not renewed, placed under probation, subject to disciplinary action or otherwise limited or curtailed; or has the organization voluntarily relinquished the accreditation in anticipation of any of these actions; or are any of these actions pending with respect to any such accreditation? ☐ **Yes** ☐ **No**
J. Has the organization ever been placed under temporary government ordered management? ☐ **Yes** ☐ **No**
K. Has the organization ever permitted the appointment of a receiver for its business or its assets? ☐ **Yes** ☐ **No**

SECTION V: LIABILITY INSURANCE INFORMATION

☐ *Professional Liability Insurance - Please attach a copy of your current Professional Liability Insurance Certificate or declaration page showing the dates and the amounts of coverage.* ☐ *Not Applicable (N/A)*

Current Insurance Carrier Mercury Allied Insurance			
Policy # PL-CA-882194	Amounts of Coverage $1,000,000 Occurrence / $3,000,000 Aggregate		
Effective Date 01/01/2026		Expiration Date 12/31/2026	Years with Carrier 3
Name of Contact with Carrier Nina Foster		Phone (555) 410-8811	Email policy.service@mercuryallied.example.org

Malpractice Claims Information

☐ N/A ☐ Malpractice Actions/Sanctions

Please complete this form if you reported any malpractice actions or sanctions on your application. All questions must be answered completely. If additional sheets are required, please photocopy this page as many times as needed prior to completing. A separate sheet must be used for each malpractice action. In the event that a Malpractice Claim is not applicable please mark N/A.

Name of Patient and/or case #:
Allegation:
Date of incident: Date reported:
Location of incident: Insurance Carrier:
Additional defendants:
Claim Status: Open ☐ Attach copy of **Complaint Notice of Intent with Affidavit**.

Closed ☐ Date Closed:

If closed, indicate method of closing:
Dismissed ☐ Settled ☐ Judgment ☐

Amount of settlement or judgment:

Explain the **condition and diagnosis at time of incident, dates and description of treatment rendered and condition of patient subsequent to treatment.** If additional space is necessary, use the reverse side or attach additional sheets. Your narrative must provide adequate clinical details to allow proper evaluation by a committee of physicians.

Narrative:

_____ _____

Name (*Please Print or type*) **Signature & Date**

SECTION VI: APPLICABLE STAFF SCREENING

Will your organization be utilizing medically licensed provider staff to provide Community Supports?

- ☐ No: Our CS Organization does not employ any of the below licensed staff for this Community
- ☐ Yes*: Our CS Organization does employ the below licensed staff. Check all applicable boxes.
  - ☐ Physician Providers: Medical Doctor (MD), Doctor of Osteopathy (DO), Doctor of Podiatric Medicine (DPM), Doctor of Dental Surgery (DDS/DMD), Psychiatrist (MD),
  - ☐ Non-Physician Medical Practitioner: Physician Assistant (PA), Nurse Practitioner (NP), Certified Nurse Midwife,
  - ☐ Allied Providers: Doctor of Chiropractic (DC), Physical Therapist (PT), Occupational Therapist (OT), Speech and Language Therapist (SLP), Acupuncturist, Licensed Midwife, Optometrist (OD),

I attest that background screening has been performed on all CS staff and that:

- ☐ Staff are physically capable of performing services.
- ☐ Staff have initial competency for specific skills required to perform job duties.
- ☐ Staff received Tuberculosis (PPD) screening and passed.
- ☐ Staff received OSHA Blood Borne Pathogens training.
- ☐ Staff received Confidentiality/HIPPA training.
- ☐ Staff received Abuse Reporting/Mandated Reporter Training

I certify that no CS Staff employed by my organization:

- ☐ Currently are excluded by a State Medicaid agency.
- ☐ Currently are excluded from any other Federal health care program.
- ☐ Currently are debarred, suspended, or otherwise excluded from participating in any other Federal procurement or non-procurement program or activity in accordance with the Federal Acquisition Streamlining Act implementing regulations and the Department of Health and Human Services non-procurement common rule at 45 CFR part 76; and
- ☐ History of fraud, waste, and/or abuse; and
- ☐ Recent history of criminal activity, including a history of criminal activities that endanger Members and/or their families.

For any boxes you are unable to check, please provide a detailed explanation on a separate sheet. For any boxes you are unable to check, please provide a detailed explanation on a separate sheet.

Signature _____ **Date** _____

*Licensed Staff as listed above are required to submit additional credentialing application requirements as outlined in PS-CR01 Provider Enrollment and Screening, PS-CR03 Provider Credentialing and Peer Review Policy and PS-CR11 Credentialing of Organizational Providers.

Please notify your Provider Services Credentialing Representative if you employ any of the listed licensed staff.

Dummy selection: No medically licensed provider staff are directly employed for CS service delivery in

this sample form.

SECTION VII: Claims and Invoice Submission

Community Supports Provider shall record, generate, and send a claim for Community Supports rendered in the standard format (837 file) **OR** shall send an invoice in a DHCS-specified format.

1) Describe your current process for recording, generating, and submitting claims or invoices for payment of services rendered. Indicate any relevant electronic systems or platforms you currently use.
   Services are documented in the care management platform, reviewed weekly, and submitted electronically through a clearinghouse and payer portal.

2) If you do not have a current process, indicate how you plan to submit claims or invoices for the Community Supports you are interested in providing.
   Not applicable for this dummy example because an active claims workflow is already in place.

3) State which format you intend to use to submit claims for CS services (e.g., Electronic 834, CMS 1500, UB04, Invoice Template, Web Direct Entry (ILS Provider Portal)):
   Primary format: electronic 837/portal submission; backup: invoice template.

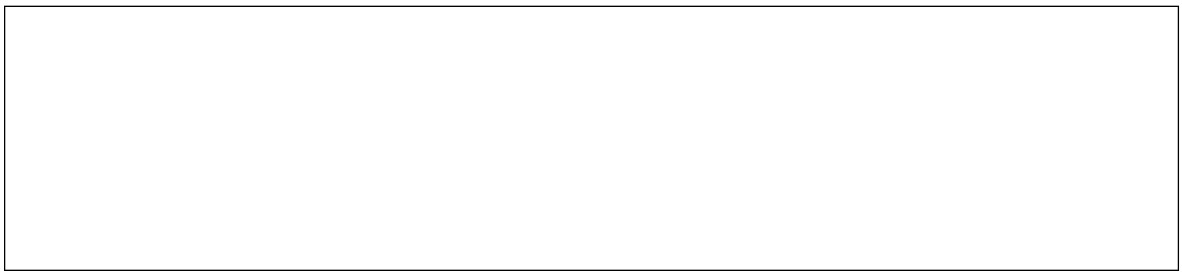
4) Describe your organization's ability to transfer data and reports with ILS via SFTP site or other secure data exchange mechanism to support service delivery.
Our IT team supports encrypted SFTP, secure portal uploads, and scheduled reporting.

5) Describe what data exchange platforms your organization currently uses.
Secure ShareFile portal, Athenahealth exports, and managed SFTP.

6) Describe your process for developing reports on Community Supports service delivery.
We create monthly service, outreach, and utilization reports from the program dashboard and QA review tracker.

7) Describe how you currently meet Privacy/Security/HIPAA requirements to provide services and prevent data breeches.
We use role-based access, device encryption, secure email, annual HIPAA training, and incident response workflows.

8) If you do not have a current process, describe how you plan to meet this requirement and what assistance you may need.
Not applicable.



ATTACHMENT A: ADDITIONAL SERVICE LOCATIONS

Location No. 3

Business Name: Blue Horizon Outreach Site 3		Phone #: (555) 310-2430	Fax #: (555) 310-2431
Street Address: 103 Main Street		Suite #: 100	City: Sacramento
State: CA	Zip: 95813	County: Sacramento	
NPI #: 1987654323	Medicaid #: MC-472983	Federal Tax ID #: 84-3179265	
Normal Business Hours: 9:00 AM - 4:30 PM		Schedule: (Check all that apply to this location) M ☒T ☒W ☒TH ☒F ☒	

Location No. 4

Business Name: Blue Horizon Outreach Site 4		Phone #: (555) 310-2440	Fax #: (555) 310-2441
Street Address: 104 Main Street		Suite #: 100	City: Sacramento
State: CA	Zip: 95814	County: Sacramento	
NPI #: 1987654324	Medicaid #: MC-472984	Federal Tax ID #: 84-3179265	
Normal Business Hours: 9:00 AM - 4:30 PM		Schedule: (Check all that apply to this location) M ☒T ☒W ☒TH ☒F ☒	

Location No. 5

Business Name: Blue Horizon Outreach Site 5		Phone #: (555) 310-2450	Fax #: (555) 310-2451
Street Address: 105 Main Street		Suite #: 100	City: Sacramento
State: CA	Zip: 95815	County: Sacramento	
NPI #: 1987654325	Medicaid #: MC-472985	Federal Tax ID #: 84-3179265	
Normal Business Hours: 9:00 AM - 4:30 PM		Schedule: (Check all that apply to this location) M ☒T ☒W ☒TH ☒F ☒	

ATTACHMENT B: COMMUNITY SUPPORTS DETAIL

For each Community Support selected in Section I complete the applicable Section(s) on the pages that follow. You are only required to complete the sections that correspond to the services you plan to offer.

1. Housing Transition Navigation Services

Licensing/Allowable Providers – please identify your provider type (select only 1):

- ☐ Vocation services agency
- ☐ Homeless services provider organization (e.g., supportive housing provider)
- ☐ Life skills training and education provider
- ☐ County agency, including county behavioral health agency
- ☐ Public hospital system
- ☐ Mental health or substance use disorder treatment provider
- ☐ Social services agency
- ☐ Affordable housing provider
- ☐ Federally qualified health centers and rural health clinics
- ☐ Public housing agency

Providers must assist Managed Care Plan members with obtaining housing and have the ability to provide all of the 15 services listed below.

Please indicate which of the following services you currently provide (check all that apply):

1. ☐ Conducting a tenant screening and housing assessment that identifies the participant's preferences and barriers related to successful tenancy. The assessment may include collecting information on the participant's housing needs, potential housing transition barriers, and identification of housing retention barriers.
2. ☐ Developing an individualized housing support plan based upon the housing assessment that addresses identified barriers, includes short- and long-term measurable goals for each issue, establishes the participant's approach to meeting the goal, and identifies when other providers or services, both reimbursed and not reimbursed by Medi-Cal, may be required to meet the goal.
3. ☐ Searching for housing and presenting options
4. ☐ Assisting in securing housing, including the completion of housing applications and securing required documentation (e.g., Social Security card, birth certificate, prior rental history)
5. ☐ Assisting with benefits advocacy, including assistance with obtaining identification and documentation for SSI eligibility and supporting the SSI application process. Such a service can be subcontracted out to retain needed specialized skillset.
6. ☐ Identifying and securing available resources to assist with subsidizing rent (such as Section 8, state and local assistance programs, etc.) and matching available rental subsidy resources to members.
7. ☐ If included in the housing support plan, identifying and securing resources to cover expenses, such as security deposit, moving costs, adaptive aids, environmental modifications, moving costs, and other one-time expenses. (Note: Actual payment of these housing deposits and move-in expenses is a separate in-lieu service under Housing Deposits.)
8. ☐ Assisting with requests for reasonable accommodation, if necessary
9. ☐ Landlord education and engagement
10. ☐ Ensuring that the living environment is safe and ready for move-in
11. ☐ Communicating and advocating on behalf of the client with landlords.
12. ☐ Assisting in arranging for and supporting the details of the move.
13. ☐ Establishing procedures and contacts to retain housing, including developing a housing support crisis plan that includes prevention and early intervention services when housing is jeopardized. (Note: The services associated with the crisis plan are a separate in-lieu service under Housing Tenancy and Sustaining Services.)
14. ☐ Identifying, coordinating, securing, or funding non-emergency, non-medical transportation to assist members' mobility to ensure reasonable accommodations and access to housing options prior to transition and on move in day.
15. ☐ Identifying, coordinating, environmental modifications to install necessary accommodations for

accessibility.

16. Are all these services provided directly in-house by your organization or are some of these referred out or subcontracted to another organization?
    All services in this sample are provided primarily in-house, with specialty referrals only when clinically indicated.
    a. ☐ All in-house.
    b. ☐ Some Referred out.
    c. ☐ Some subcontracted/purchased.

17. If not all in-house, please explain.
    Occasional specialty referrals may be used for highly specialized services; core program services remain in-house.

In accordance with DHCS Community Supports Provider Requirements, each CS Provider shall have experience and/or training in the provision of the Community Support(s) being offered and have the capacity to provide the Community Support(s) in a culturally and linguistically competent manner, as demonstrated by a successful history of providing such services, training or other factors.

18. How long has your organization been providing this service?
    5 years

19. Does your organization have an HMIS read/write account?
    Yes

STAFF

This list is meant to encompass all staff within your organization that perform this service, ILS may request additional information regarding individuals if needed. As an alternative to filling out the table below, your organization may submit a staff roster.

Last, First Name	Title	License Number	License Type	Language(s) Spoken	Counties Servicing	Open Capacity Number
Nguyen, Lara	Care Coordinator	CA-CC-2041	CHW/CC	English, Spanish	Sacramento, Yolo	18
Jackson, Omar	Program Supervisor	RN-557812	RN	English	Sacramento	12
Singh, Priya	Housing Navigator	CHW-9921	CHW	English, Hindi, Punjabi	Sacramento, San Joaquin	20
Lopez, Elena	Case Manager	LCSW-41882	LCSW	English, Spanish	Sacramento, Yolo	14
Kim, Daniel	Community Liaison	CHW-8455	CHW	English, Korean	Sacramento	16
Brown, Alexis	Operations Lead	N/A	Admin	English	Sacramento	10

Rivera, Mateo	Outreach Specialist	CHW-7410	CHW	English, Spanish	Sacramento, Placer	15

2. Housing Tenancy and Sustaining Services

Licensing/Allowable Providers – please identify your provider type (select only 1):

- ☐ Vocation services agency
- ☐ Homeless services provider organization (e.g., supportive housing provider)
- ☐ Life skills training and education provider
- ☐ County agency, including county behavioral health agency
- ☐ Public hospital system
- ☐ Mental health or substance use disorder treatment provider
- ☐ Social services agency
- ☐ Affordable housing provider
- ☐ Federally qualified health centers and rural health clinics
- ☐ CoC affiliated entities
- ☐ Public housing agency

Providers must offer tenancy and sustaining services, with a goal of maintaining safe and stable tenancy once housing is secured. Providers must have the ability to provide all 13 of the services listed below.

Please indicate which of the following services you currently provide (check all that apply):

1. ☐ Providing early identification and intervention for behaviors that may jeopardize housing, such as late rental payment, hoarding, substance use, and other lease violations.
2. ☐ Education and training on the role, rights and responsibilities of the tenant and landlord
3. ☐ Coaching on developing and maintaining key relationships with landlords/property managers with a goal of fostering successful tenancy
4. ☐ Coordination with the landlord and case management provider to address identified issues that could impact housing stability.
5. ☐ Assistance in resolving disputes with landlords and/or neighbors to reduce risk of eviction or other adverse action including developing a repayment plan or identifying funding in situations in which the client owes back rent or payment for damage to the unit.
6. ☐ Advocacy and linkage with community resources to prevent eviction when housing is or may potentially become jeopardized.
7. ☐ Assisting with benefits advocacy, including assistance with obtaining identification and documentation for SSI eligibility and supporting the SSI application process. Such service can be subcontracted out to retain the needed specialized skillset.
8. ☐ Assistance with the annual housing recertification process
9. ☐ Coordinating with the tenant to review, update and modify their housing support and crisis plan on a regular basis to reflect current needs and address housing retention barriers.
10. ☐ Continuing assistance with lease compliance, including ongoing support with activities related to household management.
11. ☐ Health and safety visits, including unit habitability inspections.
12. ☐ Other prevention and early intervention services identified in the crisis plan that are activated when housing is jeopardized (e.g., assisting with reasonable accommodation requests that were not initially required upon move-in)
13. ☐ Providing independent living and life skills including assistance with and training on budgeting, including financial literacy and connection to community resources
14. Are all these services provided directly in-house by your organization or are some of these referred out or subcontracted to another organization?

All services in this sample are provided primarily in-house, with specialty referrals only when clinically indicated.

a. ☐ All in-house.
b. ☐ Some Referred out.
c. ☐ Some subcontracted/purchased.

15. If not all in-house, please explain.
Occasional specialty referrals may be used for highly specialized services; core program services remain in-house.

In accordance with DHCS Community Supports Provider Requirements, each CS Provider shall have experience and/or training in the provision of the Community Support(s) being offered and have the capacity to provide the Community Support(s) in a culturally and linguistically competent manner, as demonstrated by a successful history of providing such services, training or other factors.

16. How long has your organization been providing this service?
5 years

17. Does your organization have an HMIS read/write account?
Yes

STAFF

This list is meant to encompass all staff within your organization that perform this service, ILS may request additional information regarding individuals if needed. As an alternative to filling out the table below, your organization may submit a staff roster.

Last, First Name	Title	License Number	License Type	Language(s) Spoken	Counties Servicing	Open Capacity Number
Nguyen, Lara	Care Coordinator	CA-CC-2041	CHW/CC	English, Spanish	Sacramento, Yolo	18
Jackson, Omar	Program Supervisor	RN-557812	RN	English	Sacramento	12
Singh, Priya	Housing Navigator	CHW-9921	CHW	English, Hindi, Punjabi	Sacramento, San Joaquin	20
Lopez, Elena	Case Manager	LCSW-41882	LCSW	English, Spanish	Sacramento, Yolo	14
Kim, Daniel	Community Liaison	CHW-8455	CHW	English, Korean	Sacramento	16
Brown, Alexis	Operations Lead	N/A	Admin	English	Sacramento	10
Rivera, Mateo	Outreach Specialist	CHW-7410	CHW	English, Spanish	Sacramento, Placer	15

3. Housing Deposits

Licensing/Allowable Providers for Housing Deposit– please identify your provider type (select only 1):

- ☐ County agency, including county behavioral health agency
- ☐ Flex Pools
- ☐ Homeless services provider organizations (e.g., supportive housing providers)
- ☐ Social services agency
- ☐ Affordable housing provider
- ☐ Public housing agency
- ☐ Other providers of services for individuals experiencing homelessness

Providers must assist with identifying, coordinating, securing, or funding one-time services and modifications necessary to enable a Managed Care Plan member to establish a basic household that does not constitute room and board. Providers must have the ability to provide all 6 of the services for Housing Deposits.

Please indicate which of the following services you currently provide (check all that apply):

1. ☐ Security deposits are required to obtain a lease on an apartment or home.
2. ☐ Set-up fees/deposits for utilities or service access and utility arrearages
3. ☐ First month coverage of utilities, including but not limited to telephone, gas, electricity, heating, and water.
4. ☐ First months and last month's rent as required by landlord for occupancy.
5. ☐ Services necessary for the individual's health and safety, such as pest eradication and one-time cleaning prior to occupancy
6. ☐ Goods such as an air conditioner or heater, and other medically necessary adaptive aids and services, designed to preserve an individuals' health and safety in the home such as hospital beds, Hoyer lifts, air filters, specialized cleaning or pest control supplies etc., that are necessary to ensure access and safety for the individual upon move-in to the home.
7. Are all these services provided directly in-house by your organization or are some of these referred out or subcontracted to another organization?
   All services in this sample are provided primarily in-house, with specialty referrals only when clinically indicated.
   a. ☐ All in-house.
   b. ☐ Some Referred out.
   c. ☐ Some subcontracted/purchased.
8. If not all in-house, please explain.
   Occasional specialty referrals may be used for highly specialized services; core program services remain in-house.

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9. How long has your organization been providing this service?

5 years

10. Does your organization have an HMIS read/write account?
    Yes
    .

STAFF

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Kim, Daniel	Community Liaison	CHW-8455	CHW	English, Korean	Sacramento	16
Brown, Alexis	Operations Lead	N/A	Admin	English	Sacramento	10
Rivera, Mateo	Outreach Specialist	CHW-7410	CHW	English, Spanish	Sacramento, Placer	15

4. Short-Term Post Hospitalization

Licensing/Allowable Providers – please identify your provider type (select only 1):

- ☐ Interim housing facilities with additional on-site support
- ☐ Shelter beds with additional on-site support
- ☐ Converted homes with additional on-site support
- ☐ Peer respite setting
- ☐ County directly operated or contracted recuperative care facilities
- ☐ Supportive housing providers
- ☐ County agencies
- ☐ Public hospital systems
- ☐ Social service agencies
- ☐ Providers of services for individuals experiencing homelessness

Providers must provide Managed Care Plan members who do not have a residence and who have high medical or behavioral health needs with the opportunity to continue their medical/psychiatric/substance use disorder recovery immediately after exiting an inpatient hospital (either acute or psychiatric or Chemical Dependency and Recovery hospital), residential substance use disorder treatment or recovery facility, residential mental health treatment facility, correctional facility, nursing facility, or recuperative care. Providers must have the ability to provide all 3 of the services listed below.

Please indicate which of the following services you currently provide (check all that apply):

1. ☐ Ongoing supports necessary for recuperation and recovery such as gaining (or regaining) the ability to perform activities of daily living, receiving necessary medical/psychiatric/substance use disorder care, case management and beginning to access other housing supports such as Housing Transition Navigation Services
2. ☐ Conduct a housing assessment.
3. ☐ Develop individualized housing support plan to identify preferences and barriers related to successful housing tenancy after Short-term Post-hospitalization Housing.
4. Are all services provided directly in-house by your organization or are some of the referred out or subcontracted to another organization?
   Services are primarily delivered in-house for this dummy sample.
   a. ☐ All in-house.
   b. ☐ Some Referred out.
   c. ☐ Some subcontracted/purchased.
5. Are all these services provided directly in-house by your organization or are some of these referred out or subcontracted to another organization?
   All services in this sample are provided primarily in-house, with specialty referrals only when clinically indicated.
   a. ☐ All in-house.
   b. ☐ Some Referred out.
   c. ☐ Some subcontracted/purchased.
6. If not all in-house, please explain.
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capacity to provide the Community Support(s) in a culturally and linguistically competent manner, as demonstrated by a successful history of providing such services, training or other factors.

7. How long has your organization been providing this service?
   5 years

8. Does your organization participate in the Coordinated Entry System (CES):
   Yes

9. Does your organization have an HMIS read/write account?
   Yes

STAFF

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5. Recuperative Care (Medical Respite)

Licensing/Allowable Providers – please identify your provider type (select only 1):

- ☐ Interim housing facilities with additional on-site support
- ☐ Shelter beds with additional on-site support
- ☐ Converted homes with additional on-site support
- ☐ Peer respite setting
- ☐ County directly operated or contracted recuperative care facilities

Providers must provide short-term residential care for individuals who no longer require hospitalization, but still need to heal from an injury or illness (including behavioral health conditions) and whose condition would be exacerbated by an unstable living environment. Providers must have the ability to provide all 6 of the services listed below.

Please indicate which of the following services you currently provide (check all that apply):

1. ☐ Interim housing with a bed and meals and ongoing monitoring of the individual's ongoing medical or behavioral health condition (e.g., monitoring of vital signs, assessments, wound care, medication monitoring).
2. ☐ Limited or short-term assistance with Instrumental Activities of Daily Living and/or ADLs
3. ☐ Coordination of transportation to post-discharge appointments
4. ☐ Connection to any other on-going services an individual may require including mental health and substance use disorder services.
5. ☐ Support in accessing benefits and housing.
6. ☐ Gaining stability with case management relationships and programs
7. Are all these services provided directly in-house by your organization or are some of these referred out or subcontracted to another organization?
   All services in this sample are provided primarily in-house, with specialty referrals only when clinically indicated.
   a. ☐ All in-house.
   b. ☐ Some Referred out.
   c. ☐ Some subcontracted/purchased.
8. If not all in-house, please explain.
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9. How long has your organization been providing this service?
   5 years

10. Does your organization participate in the Coordinated Entry System (CES)?:
    Yes

11. Does your organization have an HMIS read/write account?
    Yes

STAFF

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6. Sobering Centers

Licensing/Allowable Providers – please identify your provider type (select only 1):

☐ Sobering Center

☐ Substance use disorder facility

Sobering centers are facilities for individuals who are found to be publicly intoxicated (due to alcohol and/or other drugs) who would otherwise be transported to hospital emergency departments or jail. The centers provide people, primarily those who are homeless or in unstable living situations, with a safe, supportive environment to become sober. Sobering centers provide a variety of services including medical triage, lab testing, temporary shelter, rehydration and food service, treatment for nausea, wound and dressing changes, shower and laundry facilities, substance use education, counseling and navigation to additional substance use or other necessary health care services, and homeless care support services.

Providers are required to engage in the 4 items listed below.

Please indicate which of the following services you currently provide (check all that apply):

1. ☐ When utilizing this service, direct coordination with the county behavioral health agency is required and warm hand-offs for additional behavioral health services are strongly encouraged.
2. ☐ The service also includes screening and linkage to ongoing supportive services such as follow-up mental health and substance use disorder treatment and housing options, as appropriate.
3. ☐ This service requires partnership with law enforcement, emergency personnel, and outreach teams to identify and divert individuals to Sobering Centers. Sobering centers must be prepared to identify clients with emergent physical health conditions and arrange transport to a hospital or appropriate source of medical care.
4. ☐ The services provided should utilize best practices for clients who are homeless and who have complex health and/or behavioral health conditions including Housing First, Harm Reduction, Progressive Engagement, Motivational Interviewing, and Trauma Informed Care.
5. Are all these services provided directly in-house by your organization or are some of these referred out or subcontracted to another organization?
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   c. ☐ Some subcontracted/purchased.
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7. How long has your organization been providing this service?
   5 years

STAFF

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7. Respite Services

Licensing/Allowable Providers – please identify your provider type (select only 1):

- ☐ Home health agency
- ☐ Adult Family Home/Family Teaching Home
- ☐ Certified Family Homes for Children
- ☐ County Agencies
- ☐ Residential Care Facility for the Elderly (RCFE)
- ☐ Child Day Care Facility; Child Day Care Center; Family Child Care Home
- ☐ Respite Facility; Residential Facility: Small Family Homes (Children Only)
- ☐ Respite Facility; Residential Facility: Foster Family Agency (FFA)-Certified Family Homes (Children Only)
- ☐ Respite Facility; Residential Facility: Adult Residential Facilities (ARF)
- ☐ Respite Facility; Residential Facility: Group Homes (Children Only)
- ☐ Respite Facility; Residential Facility: Family Home Agency (FHA): Adult Family Home (AFH)/Family Teaching Home (FTH)
- ☐ Respite Facility; Residential Facility: Adult Residential Facility for Persons with Special Health Care Needs
- ☐ Respite Facility; Residential Facility: Foster Family Homes (FFHs) (Children Only)
- ☐ Short-term Residential Therapeutic Program Providers or other care providers who are serving youth with complex needs
- ☐ Community-Based Adult Services (CBAS) Facilities/Providers

Providers must provide respite services to caregivers of MCP members who require intermittent temporary supervision. The services are provided on a short-term basis because of the absence or need for relief of those persons who normally care for and/or supervise them and are non-medical in nature. Respite services can be provided in-home or in an approved out-of-home location.

Please indicate which of the following services you currently provide (check all that apply):

1. ☐ Services provided by the hour on an episodic basis because of the absence of or need for relief for those people normally providing care to individuals.
2. ☐ Services provided by the day/overnight on a short-term basis because of the absence of or need for relief for those persons normally providing the care to individuals.
3. ☐ Services that attend to the participant's basic self-help needs and other activities of daily living, including interaction, socialization and continuation of usual daily routines that would ordinarily be performed by those persons who normally care for and/or supervise them.
4. Are all these services provided directly in-house by your organization or are some of these referred out or subcontracted to another organization?
   All services in this sample are provided primarily in-house, with specialty referrals only when clinically indicated.
   a. ☐ All in-house.
   b. ☐ Some Referred out.
   c. ☐ Some subcontracted/purchased.
5. If not all in-house, please explain.
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6. How long has your organization been providing this service?
   5 years

STAFF

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8. Day Habilitation Programs

Licensing/Allowable Providers – please identify your provider type (select only 1):

☐ County agency, including county behavioral health agency
☐ Mental health or substance use disorder treatment provider
☐ Community-based providers, including federally qualified health centers
☐ Vocational skills agency
☐ Licensed clinicians with capacity and training in providing Day Habilitation Services to eligible Members, including Licensed Psychologists, Licensed Certified Social Workers, and Registered Nurses
☐ Home health agency
☐ Professional fiduciary

Providers can provide the program in a member's home or an out-of-home, non-facility setting. The programs are designed to assist the participant in acquiring, retaining, and improving self-help, socialization, and adaptive skills necessary to reside successfully in the person's natural environment. The services are often considered as peer mentoring when provided by an unlicensed caregiver with the necessary training and supervision. Providers must have the ability to provide all 13 of the services listed below. Indicate (1) which of the services your organization currently provides to clients; and (2) whether the service is provided in-house by your organization, referred to another provider(s), or subcontracted to/purchased from other provider(s). Indicate which of the examples of Day Habilitation you currently provide to clients. Check all that apply:

1. ☐ Selecting and moving into a home
2. ☐ Locating and choosing suitable housemates In-house Referral
3. ☐ Locating household furnishings
4. ☐ Settling disputes with landlords
5. ☐ Managing personal financial affairs
6. ☐ Recruiting, screening, hiring, training, supervising, and dismissing personal attendants
7. ☐ Dealing with and responding appropriately to governmental agencies and personnel
8. ☐ Asserting civil and statutory rights through self-advocacy
9. ☐ Building and maintaining interpersonal relationships, including a circle of support
10. ☐ Coordination with Medi-Cal managed care plan to link participant to any Community Supports and/or enhanced care management services for which the client may be eligible.
11. ☐ Referral to non-Community Supports housing resources if participant does not meet Community Supports eligibility criteria for Housing Transition/Navigation Services
12. ☐ Assistance with income and benefits advocacy including general assistance/general relief and SSI if client is not receiving these services through Community Supports eligibility criteria or enhanced care management.
9. ☐ Coordination with Medi-Cal MCP to link participant to health care, mental health services, and substance use disorder services based on the individual needs of the participant for participants who are not receiving this linkage through Community Supports or enhanced care management
10. Are all these services provided directly in-house by your organization or are some of these referred out or subcontracted to another organization?
    All services in this sample are provided primarily in-house, with specialty referrals only when clinically indicated.
    a. ☐ All in-house.
    b. ☐ Some Referred out.
    c. ☐ Some subcontracted/purchased.
11. If not all in-house, please explain.

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12. How long has your organization been providing this service?
    5 years

STAFF

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9) Nursing Facility Transition/ Diversion to Assisted Living Facilities

Licensing/Allowable Providers – please identify your provider type (select only 1):

- ☐ Case Management agencies
- ☐ Home Health agencies
- ☐ ARF/RCFE Operators
- ☐ 1915c Home & Community Based Alternatives (HCBAs)/ALW providers
- ☐ CCT/Money Follows the Person providers

Providers are required to assist members with living in the community and/or avoid institutionalization, when possible, with the goal being to both facilitate nursing facility transition back into a home-like, community setting and/or prevent skilled nursing admissions for beneficiaries with an imminent need for nursing facility level of care (LOC). Providers must have the ability to provide all six of the services listed below. Indicate (1) which of the services your organization currently provides to clients; and (2) whether the service is provided in- house by your organization, referred to another provider(s), or subcontracted to/purchased from other provider(s)

Indicate which of the examples of Nursing Facility Trans. / Div. to Assisted Living you currently provide to clients. Check all that apply:

1. ☐ Assessing the participant's housing needs and presenting options
2. ☐ Assessing the service needs of the participant to determine if the participant needs enhanced onsite services at the RCFE/ARF so the client can be safely and stably housed in an RCFE/ ARF
3. ☐ Assisting in securing a facility residence, including the completion of facility applications and securing required documentation (e.g., Social Security card, birth certificate, prior rental history)
4. ☐ Communicating with facility administration and coordinating the move
5. ☐ Establishing procedures and contacts to retain facility housing.
6. ☐ Coordinating with the Medi-Cal plan to ensure that the needs of participants who need enhanced services to be safely and stably housed in RCFE/ARF settings have Community Supports and/or Enhanced Care Management services that provide the necessary enhanced services or fund RCFE/ARF operator directly to provide enhanced services.
7. Are all these services provided directly in-house by your organization or are some of these referred out or subcontracted to another organization?
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9. How long has your organization been providing this service?
   5 years

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10) Community Transition Services/ Nursing Facility Transition to a Home

Licensing/Allowable Providers – please identify your provider type (select only 1):

- ☐ Case management agencies
- ☐ Home Health agencies
- ☐ County-operated or county-contracted behavioral health providers
- ☐ 1915c HCBA/ALW providers
- ☐ CCT/Money Follows the Person providers

Providers must help members to live in the community and avoid further institutionalization. Allowable expenses are those necessary to enable a person to establish a basic household that do not constitute room and board. Providers must have the ability to provide all seven services listed below. Indicate (1) which of the services your organization currently provides to clients; and (2) whether the service is provided in-house by your organization, referred to another provider(s), or subcontracted to/purchased from other provider(s).

Check all that apply:

1. ☐ Assessing the participant's housing needs and presenting options
2. ☐ Assisting in searching for and securing housing, including the completion of housing applications and securing required documentation (e.g., Social Security card, birth certificate, prior rental history)
3. ☐ Establishing procedures and contacts to retain housing.
4. ☐ Identifying, coordinating, securing, or funding non- emergency, non-medical transportation to assist members' mobility to ensure reasonable accommodations and access to housing options prior to transition and on move-in day.
5. ☐ Identifying the need for and coordinating funding for environmental modifications to install necessary accommodations for accessibility.
6. ☐ Identifying the need for and coordinating funding for services and modifications necessary to enable a person to establish a basic household that does not constitute room and board, such as: security deposits required to obtain a lease on an apartment or home; set-up fees for utilities or service access; first month coverage of utilities, including telephone, electricity, heating and water; services necessary for the individual's health and safety, such as pest eradication and one-time cleaning prior to occupancy; home modifications, such as an air conditioner or heater; and other medically- necessary services, such as hospital beds, Hoyer lifts, etc. to ensure access and reasonable accommodations
7. Are all these services provided directly in-house by your organization or are some of these referred out or subcontracted to another organization?
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9. How long has your organization been providing this service?
   5 years

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11) Personal Care and Homemaker Services

Licensing/Allowable Providers – please identify your provider type (select only 1):

- ☐ Home health agencies
- ☐ County agencies
- ☐ Personal care agencies
- ☐ AAA (Area Agency on Aging)

Personal Care and Homemaker Services are provided for individuals who need assistance with Activities of Daily living (ADL) such as bathing, dressing, toileting, ambulation or feeding. Personal Care services can also include assistance with Instrumental Activities of Daily living (IADL) such as meal preparation, grocery shopping and money management. This ILOS should only be utilized if appropriate and if additional hours/supports are not authorized by IHSS.

Please indicate which of the following services you currently provide (check all that apply):

1. ☐ Assistance with Activities of Daily Living (ADL) such as bathing, dressing, toileting, ambulation or feeding.
2. ☐ Assistance with Instrumental Activities of Daily Living (IADL) such as meal preparation, grocery shopping and money management
3. ☐ Housecleaning and laundry
4. ☐ Accompaniment to medical appointments
5. ☐ Protective supervision for the mentally impaired
6. Are all these services provided directly in-house by your organization or are some of these referred out or subcontracted to another organization?
   All services in this sample are provided primarily in-house, with specialty referrals only when clinically indicated.
   a. ☐ All in-house.
   b. ☐ Some Referred out.
   c. ☐ Some subcontracted/purchased.
7. If not all in-house, please explain.
   Occasional specialty referrals may be used for highly specialized services; core program services remain in-house.

In accordance with DHCS Community Supports Provider Requirements, each CS Provider shall have experience and/or training in the provision of the Community Support(s) being offered and have the capacity to provide the Community Support(s) in a culturally and linguistically competent manner, as demonstrated by a successful history of providing such services, training or other factors.

8. How long has your organization been providing this service?
   5 years

STAFF

This list is meant to encompass all staff within your organization that perform this service, ILS may request additional information regarding individuals if needed. As an alternative to filling out the table below, your

organization may submit a staff roster.

Last, First Name	Title	License Number	License Type	Language(s) Spoken	Counties Servicing	Open Capacity Number
Nguyen, Lara	Care Coordinator	CA-CC-2041	CHW/CC	English, Spanish	Sacramento, Yolo	18
Jackson, Omar	Program Supervisor	RN-557812	RN	English	Sacramento	12
Singh, Priya	Housing Navigator	CHW-9921	CHW	English, Hindi, Punjabi	Sacramento, San Joaquin	20
Lopez, Elena	Case Manager	LCSW-41882	LCSW	English, Spanish	Sacramento, Yolo	14
Kim, Daniel	Community Liaison	CHW-8455	CHW	English, Korean	Sacramento	16
Brown, Alexis	Operations Lead	N/A	Admin	English	Sacramento	10
Rivera, Mateo	Outreach Specialist	CHW-7410	CHW	English, Spanish	Sacramento, Placer	15

12) Environmental Accessibility Adaptations (Home Modifications)

Licensing/Allowable Providers – please identify your provider type (select only 1):

☐ Area Agencies on Aging (AAA)
☐ Local health departments
☐ Community-based providers and organizations

Providers must make physical adaptations to a home that are necessary to ensure the health, welfare, and safety of the individual, or enable MCP members to function with greater independence in the home, without which the member would require institutionalization. Providers must have the ability to provide all of the services listed below.

Please indicate which of the examples of home modifications that you currently provide to clients. Check all that apply:

1. ☐ Physical or occupational therapy evaluation and report to evaluate the medical necessity.
2. ☐ Obtain a minimum of two bids from appropriate providers for the requested service.
3. ☐ Provide home visits to determine the suitability of any requested equipment or service.
4. ☐ Ramps and grab-bars to assist beneficiaries in accessing the home.
5. ☐ The doorway is widened for beneficiaries who require a wheelchair.
6. ☐ Stair lifts
7. ☐ Making a bathroom and shower wheelchair accessible (e.g., constructing a roll-in shower)
8. ☐ Installation of specialized electric and plumbing systems that are necessary to accommodate the medical equipment and supplies of the beneficiary.
9. ☐ Installation and testing of a Personal Emergency Response System (PERS) for persons who are alone for significant parts of the day without a caregiver and who otherwise require routine supervision (including monthly service costs, as needed)
10. ☐ Other:
11. Are all these services provided directly in-house by your organization or are some of these referred out or subcontracted to another organization?
    All services in this sample are provided primarily in-house, with specialty referrals only when clinically indicated.
    a. ☐ All in-house.
    b. ☐ Some Referred out.
    c. ☐ Some subcontracted/purchased.
12. If not all in-house, please explain.
    Occasional specialty referrals may be used for highly specialized services; core program services remain in-house.

In accordance with DHCS Community Supports Provider Requirements, each CS Provider shall have experience and/or training in the provision of the Community Support(s) being offered and have the capacity to provide the Community Support(s) in a culturally and linguistically competent manner, as demonstrated by a successful history of providing such services, training or other factors.

13. How long has your organization been providing this service?
    5 years

STAFF

This list is meant to encompass all staff within your organization that perform this service, ILS may request

additional information regarding individuals if needed. As an alternative to filling out the table below, your organization may submit a staff roster.

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Rivera, Mateo	Outreach Specialist	CHW-7410	CHW	English, Spanish	Sacramento, Placer	15

13) Asthma Remediation

Licensing/Allowable Providers – please identify your provider type (select only 1):

- ☐ Lung health organizations
- ☐ Healthy housing organizations
- ☐ Local health departments
- ☐ Community-based providers and organizations

Providers must provide physical modifications to a home environment that are necessary to ensure the health, welfare, and safety of the individual, or enable the individual to function in the home and without which acute asthma episodes could result in the need for emergency services and hospitalization.

Providers must have the ability to provide all of the services listed below.

Please indicate which of the examples of asthma remediation you currently provide to clients. Check all that apply:

1. ☐ Allergen-impermeable mattress and pillow dustcovers
2. ☐ High-efficiency particulate air (HEPA) filtered vacuums.
3. ☐ Integrated Pest Management (IPM) services
4. ☐ De-humidifiers
5. ☐ Air filters
6. ☐ Other moisture-controlling interventions
7. ☐ Minor mold removal and remediation services
8. ☐ Ventilation improvements
9. ☐ Asthma-friendly cleaning products and supplies
10. ☐ Other interventions identified to be medically appropriate and cost effective.
11. Are all these services provided directly in-house by your organization or are some of these referred out or subcontracted to another organization?
    All services in this sample are provided primarily in-house, with specialty referrals only when clinically indicated.
    a. ☐ All in-house.
    b. ☐ Some Referred out.
    c. ☐ Some subcontracted/purchased.
12. If not all in-house, please explain.
    Occasional specialty referrals may be used for highly specialized services; core program services remain in-house.

In accordance with DHCS Community Supports Provider Requirements, each CS Provider shall have experience and/or training in the provision of the Community Support(s) being offered and have the capacity to provide the Community Support(s) in a culturally and linguistically competent manner, as demonstrated by a successful history of providing such services, training or other factors.

13. How long has your organization been providing this service?
    5 years

STAFF

This list is meant to encompass all staff within your organization that perform this service, ILS may request additional information regarding individuals if needed. As an alternative to filling out the table below, your organization may submit a staff roster.

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